SERENE

AESTHETICS, WELLNESS AND IV INFUSION CENTER

FREE GUIDE

Thinning Hair? Causes, Options and What to Know About PRP

How to tell normal shedding from thinning, what to check first, and what to realistically expect from PRP hair restoration

SERENE MED SPA · PHYSICIAN-LED

Hudson, Ohio · Barboursville, West Virginia

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Educational guide · not medical advice

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Noticing more hair in your brush, a wider part or more scalp in photos can be unsettling. It is also very common, in both women and men, and it often has more than one cause.

This guide walks you through it in plain English: normal shedding vs. true thinning, the most common causes, when to see a doctor first, how PRP (platelet-rich plasma) hair restoration works, what the research really says, and what it costs.

Serene Med Spa is a physician-led med spa with offices in Hudson, Ohio and Barboursville, West Virginia. Our goal is to help you make an informed choice, whatever you decide.

About Serene Med Spa

Serene is a physician-led med spa with offices in Hudson, Ohio and Barboursville, West Virginia, led by Robin Arora, MD and Shweta Arora, MD. We plan every treatment around your goals and health history, and we'll tell you plainly what a treatment can and can't do.

PART 01

Normal shedding vs. thinning: what is actually happening?

Every hair on your head goes through a cycle. It grows for several years, rests for a few months, then falls out so a new hair can take its place. Because of this cycle, losing some hair every day is normal. The American Academy of Dermatology notes that shedding about 50 to 100 hairs a day is typical.

Shedding and thinning are not the same thing. Shedding means more hairs than usual are falling out, but the follicles are still working. Thinning means hairs are growing back finer, shorter or not at all in certain areas. Some people have both at once.

- Your part looks wider, or you can see more scalp under bright light.
- Your ponytail feels smaller.
- Your hairline is moving back, or the crown looks thinner in photos.
- New hairs look finer and shorter than the rest of your hair.
- Clumps of hair come out in the shower or on your pillow for weeks.

> **Start tracking now**
>
> Take photos of your part, hairline and crown once a month in the same spot, with the same lighting and hair style. Hair changes slowly, so photos are the most reliable way to see whether things are getting worse, staying stable or improving.

PART 02

Common causes of thinning in women and men

Hair is sensitive to what is going on in the rest of your body. Many causes are temporary or treatable, which is why finding the cause comes first.

- Pattern hair loss (androgenetic alopecia): the most common cause in both men and women. It tends to run in families and develops gradually. Men often notice a receding hairline or thinning crown. Women more often notice a widening part and thinning over the top of the head, while the front hairline usually stays in place.
- Telogen effluvium (shedding after a trigger): a stressful event pushes many hairs into the resting phase at once. Common triggers include giving birth, a high fever or illness, surgery, major emotional stress, losing a lot of weight quickly and stopping birth control pills. Shedding usually starts a few months after the trigger and often settles within 6 to 9 months.
- Postpartum shedding: a common form of telogen effluvium that often shows up a few months after delivery.
- Thyroid problems: both an underactive and an overactive thyroid can cause thinning hair.

- Low iron: low iron stores, with or without anemia, are linked to hair shedding, especially in women with heavy periods.
- Hormonal shifts: perimenopause and menopause, pregnancy, and conditions that affect hormone balance can all change how hair grows.
- Tight hairstyles (traction alopecia): ponytails, braids, extensions or weaves that pull for long periods can thin the hairline over time.
- Other causes: some medicines, patchy hair loss from an immune condition (alopecia areata) and scalp conditions.

PART 03

When to see a doctor first, and which labs are commonly checked

PRP and other cosmetic treatments work on hair follicles. They do not fix a thyroid problem, low iron or an underlying illness. If something else is driving your hair loss, treating it comes first, and your hair may recover on its own.

Talk with your primary care doctor or a dermatologist before any hair treatment if you notice any of the signs below.

- Sudden, rapid or patchy hair loss, or smooth bald patches
- Redness, scaling, pain, burning, itching or scarring on the scalp
- Loss of eyebrows, eyelashes or body hair
- Fatigue, feeling cold or hot all the time, weight changes or a racing heart
- Heavy, irregular or missed periods, new acne or new facial hair
- Hair loss that started after a new medicine (do not stop any medicine without asking the doctor who prescribed it)

> **Labs doctors commonly check**
>
> Your doctor decides which tests make sense for you, but common ones include a complete blood count (CBC), iron studies including ferritin (your iron stores), and thyroid tests such as TSH. Depending on your symptoms, your doctor may also check vitamin D or hormone levels. Ask before starting supplements, since some can affect lab results or interact with medicines.

PART 04

How PRP for hair works

PRP stands for platelet-rich plasma. Platelets are tiny cells in your blood that help with clotting and healing. They also carry growth factors, which are proteins that signal cells to repair and grow.

For PRP hair restoration, a small amount of your own blood is drawn, much like a routine lab test. The blood is spun in a machine called a centrifuge, which separates the platelet-rich part from the rest. That concentrated plasma is then injected into the scalp in the thinning areas.

The idea is that the growth factors may help support hair follicles that are still active, encouraging thicker hair and a longer growing phase. Because PRP comes from your own blood, the risk of an allergic reaction is very low.

One key point: PRP works on follicles that are still alive. It does not create new ones. That is why it makes more sense for early to moderate thinning than for areas that have been bald for a long time.

PART 05

Who tends to be a good candidate, and who is not

PRP is not right for everyone. A good consultation should tell you honestly whether it is likely to help you.

- Often a good fit: women and men with early to moderate pattern thinning, such as a widening part, thinning crown or receding hairline, who still have visible fine hairs in those areas.
- Often a good fit: people who have already ruled out or treated other causes, such as thyroid or iron problems.
- Often a good fit: people who can commit to a series of sessions plus maintenance, as part of a broader plan.
- Less likely to help: areas that have been smooth and bald for years, or scarring types of hair loss, which need a dermatologist's care.
- May not be needed: recent shedding after a clear trigger, such as childbirth or illness, which often improves on its own. Your provider may suggest waiting and watching first.

PART 06

What a session is like, and how many you may need

Your first visit starts with a consultation about your hair history, health, medicines and goals. Your scalp will be examined, and photos are usually taken to measure progress.

On treatment day, a small blood sample is drawn and processed into PRP. At Serene, the scalp is numbed for comfort before the PRP is injected across the thinning areas using a fine needle. Most people feel pressure and some stinging. A full visit, including the blood draw and processing, often takes about an hour, but ask your office for its exact timing.

Downtime is usually minimal. Most people go back to their day right away. Your scalp may feel tender, tight or a little swollen for a day or two.

- A typical plan: an initial series of sessions spaced a few weeks apart. Many studies used about 3 sessions roughly a month apart.

- Then maintenance: periodic touch-up sessions, often every 3 to 6 months, to help keep the results. Your provider will suggest a schedule based on how you respond.
- Timeline: hair grows slowly, so changes usually take several months to show. Many providers reassess at about 6 months.
- Aftercare: your provider may ask you to wait a while before washing your hair and to skip hair color, heavy workouts and saunas for a short time. Follow the instructions you are given.

> **Plan for the long game**
>
> PRP is a series and maintenance commitment, not a one-time fix. If you stop, the underlying cause of pattern thinning keeps going, and gains may fade over time.

PART 07

Side effects and who should avoid PRP

PRP is generally considered low risk because it uses your own blood. Side effects are usually mild and short-lived.

- Common: pain or stinging during injections, scalp tenderness, mild swelling, redness or pinpoint bleeding, and bruising or soreness where blood was drawn.
- Sometimes: a headache, or temporary extra shedding in the weeks after treatment.
- Rare: infection at an injection site, or feeling lightheaded during the blood draw.
- Who should avoid PRP or check with a doctor first: people with platelet or bleeding disorders, low platelet counts, blood-related cancers or cancer under active treatment, a current infection or scalp infection, or serious liver disease.
- Tell your provider if you take blood thinners or anti-inflammatory pain relievers. Do not stop a prescribed medicine on your own.
- Most providers advise waiting until after pregnancy and breastfeeding.

PART 08

Realistic expectations: what the research says

PRP for hair has been studied a lot in the past decade. Several peer-reviewed systematic reviews and meta-analyses, which combine results from many studies, have found that PRP improved hair density and thickness on average in people with pattern hair loss, including studies focused on women.

That evidence is promising, but it is also mixed. Many studies are small and short. Clinics prepare and inject PRP in different ways, with different numbers of sessions, so it is hard to compare results. Some people respond well, some see a modest change, and some see little difference.

A realistic goal for most people is thicker, fuller-looking hair and less thinning over time, not a full head of hair. Results vary, and no treatment can promise a specific result.

> **How to judge whether it is working**
>
> Compare your monthly photos, not the mirror. Look for less scalp at the part or crown, more short new hairs and less shedding. Give it several months before deciding.

PART 09

How PRP fits alongside other treatments

Pattern hair loss is ongoing, and most experts favor a combined approach. PRP is often used as an add-on to medical care, not a replacement for it. Some research reviews suggest PRP plus medical treatment may work better than medical treatment alone, though more study is needed.

Medical treatments: your doctor or dermatologist may recommend medications, including topical and oral options. These have the longest track record for pattern hair loss. They are not right for everyone, especially during pregnancy, so the choice should be made with your doctor.

Low-level light therapy: caps, helmets and combs that use red light at home. Some devices are FDA-cleared for pattern hair loss. They require regular use over many months.

Hair transplant: a surgical option that moves follicles from fuller areas to thinning areas. It requires enough donor hair, has a recovery period and costs more. Ongoing treatment is often still recommended to protect the hair you have.

Needle-free option at Serene: we also offer Alma TED, a needle-free treatment that uses sound waves to help deliver a topical scalp formula. It is often chosen by people who want to avoid injections, and it can be combined with PRP. Research on it is still early, and results vary.

Hair thinning options at a glance

	PRP hair restoration	Medical treatments (doctor-prescribed or recommended)	Low-level light therapy	Hair transplant
How it works	Your own concentrated platelets are injected into the scalp to support active follicles	Topical and oral medications that act on hair growth or the hormones behind pattern loss	Red light from a cap, helmet or comb used at home	Surgery moves follicles from fuller areas to thinning areas
Often suited to	Early to moderate thinning with active follicles	Most people with pattern hair loss, if medically appropriate	Mild to moderate thinning; people who prefer at-home care	Defined bald areas with enough donor hair
Commitment	Series of sessions, then maintenance every few months	Daily use, long term	Several sessions a week for months	One or more procedures, plus ongoing care
Downtime	Minimal; scalp may be tender for a day or two	None; side effects vary by medication	None	Days to weeks of healing
Evidence	Promising but mixed; results vary	Longest track record for pattern loss	Some supportive studies; results vary	Established surgical option; results depend on donor hair and surgeon

What it costs at Serene

Treatment	Hudson, OH	Barboursville, WV
PRP Hair Restoration, per treatment Offered at both offices	$500	$500
PRP Hair Restoration, series of 3 (prepaid) $300 less than three single sessions ($1,500)	$1,200	$1,200
Alma TED (needle-free), per treatment Can be combined with PRP	$500	$500
Alma TED, series of 3 (prepaid)	$1,200	$1,200

Prices as of October 2026; see serenemedspas.com for current pricing. Prices are starting points, and your final cost is confirmed at your consultation. Example budget: a prepaid series of 3 PRP sessions ($1,200) plus 2 maintenance sessions in the first year ($1,000) comes to about $2,200. New clients: "20% off your first visit — applies to any product or service; can't be combined with another discount". For example, a single $500 PRP session would be $400.

Your consultation checklist

- [ ] Bring your monthly hair photos and a timeline of when you first noticed changes.
- [ ] List any recent triggers: childbirth, illness, surgery, major stress, weight loss or new medicines.
- [ ] Bring recent lab results, such as CBC, ferritin and thyroid tests, if you have them.
- [ ] Share all medicines and supplements you take, especially blood thinners and pain relievers.
- [ ] Ask: Do my follicles look active enough for PRP to help?
- [ ] Ask: Should I see my doctor or a dermatologist first, or at the same time?
- [ ] Ask: How many sessions do you recommend, how far apart, and what will maintenance cost each year?
- [ ] Ask: How will we measure progress, and when will we decide whether it is working?
- [ ] Ask: How can PRP fit with medications my doctor may recommend?
- [ ] Ask: What side effects should I expect, and who do I contact if I have a concern?

Frequently asked questions

Does PRP for hair hurt?

Most people feel pressure and stinging during the injections. At Serene, the scalp is numbed first for comfort. The scalp may feel tender for a day or two afterward.

How soon will I see results?
Hair grows slowly, so most changes take several months to show. Many providers reassess at about 6 months. Results vary from person to person.

Is PRP a one-time fix?
No. Pattern hair loss is ongoing, so PRP usually needs maintenance sessions every few months to help keep results. If you stop, gains may fade over time.

Does PRP work for women?
PRP is offered to both women and men. Several research reviews include studies focused on women with pattern thinning and report improvements on average, but results vary and more research is needed.

Can PRP help postpartum shedding?
Postpartum shedding often improves on its own within several months. Many providers suggest giving it time first, and most advise waiting until after breastfeeding for PRP. If shedding lasts longer than expected, talk with your doctor.

Can I use PRP along with medications?
Often, yes. PRP is commonly used alongside medications your doctor may recommend, including topical and oral options. Tell us everything you take so we can plan safely.

Should I take a hair supplement?
Please ask your doctor before starting supplements. Some can affect lab results or interact with medicines, and they may not help unless you have a true deficiency.

YOUR NEXT STEP

Ready when you are

- Book a hair consultation online: Hudson at https://serenemedspas.com/hudson/ or Barboursville at https://serenemedspas.com/barboursville/. PRP hair restoration is offered at both offices.
- Prefer to text? Hudson: (330) 303-2031. Barboursville: (304) 396-0295. Send us your questions before you book.
- New clients: "20% off your first visit — applies to any product or service; can't be combined with another discount".
- If perimenopause or other hormone changes are part of the picture, ask about Serene Telehealth hormone care by video in Ohio, West Virginia, Kentucky and Florida.



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Scan a code to see pricing and book online.

Important

This guide is for general education only and is not medical advice. It does not replace an evaluation by your doctor or a licensed medical provider. Hair loss can have medical causes that need diagnosis and treatment. PRP and other hair treatments carry risks, and results vary from person to person; no specific outcome is promised. Do not start, stop or change any medicine or supplement without talking to your doctor. Prices are as of October 2026 and may change; see serenemedspas.com for current pricing. If you have a medical emergency, call 911.

Sources

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